

# Supporting someone who is grieving

**Listen. Help with what matters. Stay connected.**

## WHO THIS IS FOR · HOW TO USE IT

For community-based MHPSS and other frontline workers trained in basic PFA, with supervision and known local referral routes. A4 front/back reminder for preparing, supporting and reviewing a visit—not diagnosis, grief therapy or a substitute for training.

## SIDE 1 · DURING THE VISIT



### 01 · Introduce your help. Ask permission.

Explain what you can and cannot offer. Ask where the person feels comfortable talking. Check safety, health, basic needs and care for anyone depending on them.

*“I can listen and help you find practical support. Would you like that?”*



### 02 · Listen—or accept silence.

Let the person speak, stay quiet or stop. Acknowledge the loss. Do not demand details, compare losses, say “be strong”, or promise when they will feel better.

*“We can talk—or sit quietly. You do not have to tell your story.”*



### 03 · Help with one chosen need.

Ask what matters most now. Help with an available practical option or a trusted connection. Ask what support or mourning practices they want; never prescribe prayer, rites, touch or disclosure.

*“What would help today? What is getting in the way?”*

Before contact: check local risks, language/access needs, your supervisor and available health, emergency and Child Protection routes. These actions are flexible—not stages of grief.

**DANGER, SELF-HARM RISK OR SERIOUS ILLNESS/INJURY? Seek health/emergency help now; alert your supervisor. If safe, stay or arrange trusted company until the responding service takes over. Do not finish these steps first. More urgent signs: Side 2.**

**SAMPLE · ENGLISH REVIEW DRAFT v3 · NOT FOR FIELD DISTRIBUTION**

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## SIDE 2 · CONNECT AND FOLLOW THROUGH

Support is guided by need—not a recovery deadline.



### 04 · When a relative is missing...

Hope and grief can coexist. Do not assume death or push for “closure”. Respect choices about waiting or mourning. Link to verified tracing/identification information. Never promise an answer or date.

*“What information or support would be useful now?”*



### 05 · Offer additional help when needed.

If distress worsens, keeps troubling them or disrupts daily life, consult your supervisor or a health worker. Explain available options, seek consent and help them reach agreed care. Do not wait six months. Physical symptoms need medical attention too.

*“Would you like help contacting a health worker?”*



### 06 · Agree the next contact. Check access.

Agree when and how to reconnect. Check whether help was received; if not, identify the barrier with your supervisor. Record only required details in the approved secure system. Explain privacy and its limits; share only as authorised.

*“Did you get the help we discussed? What made it difficult?”*

## URGENT CONCERN? ACT NOW.

Suicide/self-harm risk, danger to others, serious illness or injury, or severe confusion/distress preventing essential self-care or safe care of dependants needs urgent help.

Seek health or emergency help now and alert your supervisor. If safe, stay with the person—or arrange someone trusted to stay—until the responding health/emergency service takes responsibility. Follow the protocol you are trained to use; do not put yourself at risk.

Lack of food, shelter or caregiving support needs practical assistance; it is not, by itself, a mental health emergency.

CHILD PROTECTION · Follow your safeguarding pathway for separated or unaccompanied children, abuse or other protection concerns. Do not investigate or promise absolute secrecy.

Local contacts must be checked before use: supervisor · health/emergency · Child Protection · tracing/identification. Keep the verified contact card with this sheet.

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